

Emergency Department Ultrasound Serratus Anterior Plane Block Record

Clinician :Signature.....

Supervisor:

GMC/Reg number:

Date:

Patient details

Full
name

DoB

Unit
number

(use sticker if available)

Patient Weight.....Kg Site of Procedure LEFT ☐ RIGHT ☐ BILATERAL ☐

Max TOTAL Local Anaesthetic dose (2mg/Kg -ideal body weight), L/Bupivacaine 0.25% or 2.5mg /ml) - Maximum 60ml

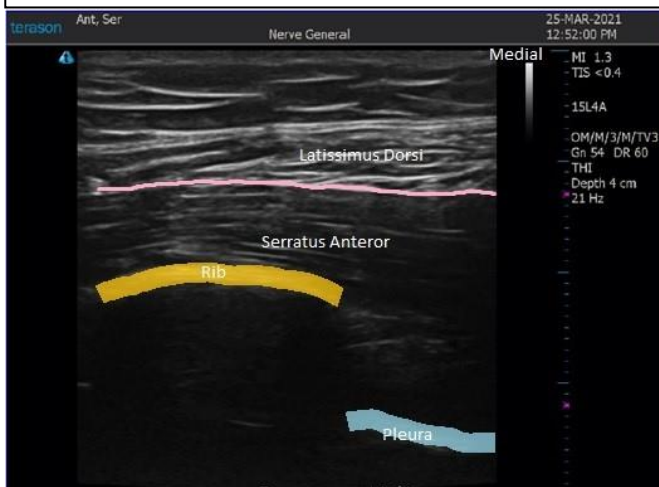
0.8ml

X

Kg

=

mls of 0.25% L/Bupivacaine



Checklist

- ☐ Patient information leaflet provided
- ☐ Consent
- ☐ Time out
 - Right Patient ☐
 - Correct side ☐
 - Max dose to be given ☐
- ☐ Aseptic technique
- ☐ Skin prep 2% Chlorhexadine
- ☐ Skin local anaesthetic 0.5mls 1% lidocaine
- ☐ 0.25% L/Bupivacaine into plane
- ☐ Ultrasound guided approach
- ☐ Observations at 5, 10, 15 & 30mins
- ☐ No complications OR
- ☐ Complications occurred:

.....
.....
.....

Quick Reference Information Serratus Anterior plane Block

Advantages:

- Technically easy and superficial block.
- Performed with patient supine.
- Suitable for patients with associated spinal trauma or head injuries where paravertebral and epidural blocks may be contraindicated.
- Can be inserted in patients with a coagulopathy.
- Can form part of an opiate sparing analgesic strategy.

Contraindications:

Absolute

- Patient refusal
- Local anaesthetic allergy
- Infection at needle insertion point

Relative

- Distorted anatomy e.g. surgical emphysema

Complications: (see also Patient Information Leaflet to be provided)

- Pneumothorax – due to puncture of the pleura
- Vascular puncture the thoracodorsal artery lies medially
- Nerve damage unlikely with a plane block however the thoracodorsal nerve does run with the thoracodorsal artery
- Failure/inadequate block
- Local anaesthetic systemic toxicity (LAST)
- Infection